

Specialty-Diagnosis Alignment Policy

Synthetic Training Policy - Non-PHI

Defines expected alignment between provider specialty and diagnosis category. Specialty-diagnosis mismatches are a standard medical-necessity gate and directly inform the `severity_procedure_mismatch` and `specialty_diagnosis_mismatch` features.

Policy ID	CLAIMOPS-POL-008
Effective Date	2026-04-25
Applies To	Synthetic claims, provider, diagnosis, and cost datasets in this repository
Source Type	Locally generated demo policy; not a real payer coverage policy

Policy Summary

- Each provider specialty has a defined set of diagnosis categories it typically manages.
- When a provider bills a procedure for a diagnosis outside their specialty alignment, `specialty_diagnosis_mismatch` is set to 1.
- A high-severity diagnosis paired with a low-intensity procedure triggers `severity_procedure_mismatch` regardless of specialty alignment.
- Specialty alignment complements, but does not replace, diagnosis-procedure compatibility review.

Rules

Rule ID	Requirement
SPEC-01	Provider specialties in this dataset: Cardiology, General, Neurology, Orthopedic. Each specialty has one or more aligned diagnosis categories: Cardiology aligns with D10 Heart and D50 Diabetes (cardiac complications); Orthopedic aligns with D20 Bone; Neurology aligns with D10 Heart (neuro-cardiac) and D50 Diabetes (neuropathy); General aligns with D30 Fever, D40 Skin, D60 Cold (low-severity acute care).
SPEC-02	When provider specialty does not align with the diagnosis category, <code>specialty_diagnosis_mismatch</code> is set to 1. Mismatched claims require documentation explaining why a non-aligned specialist performed the service.
SPEC-03	A provider with missing or unknown specialty should be treated as <code>specialty_diagnosis_mismatch = 1</code> for conservative risk assessment. Confirm specialty before submitting the claim to the payer.

Remediation Guidance

- Verify the provider's specialty against the billed diagnosis before payer submission.
- If specialty mismatch is identified, request documentation from the referring or supervising physician.
- If the diagnosis was coded in error, correct the `diagnosis_code` and re-run validation.
- For legitimate cross-specialty care (e.g., cardiologist managing diabetes complications), attach a supporting note.

RAG Safety Note

This document contains only synthetic policy text. It intentionally avoids patient names, patient identifiers, service dates, and real billed amounts. Use policy_id, rule_id, denial reason, diagnosis category, and procedure status as retrieval metadata.